

Patient ID: EG

Period: March 26 to April 25, 2024

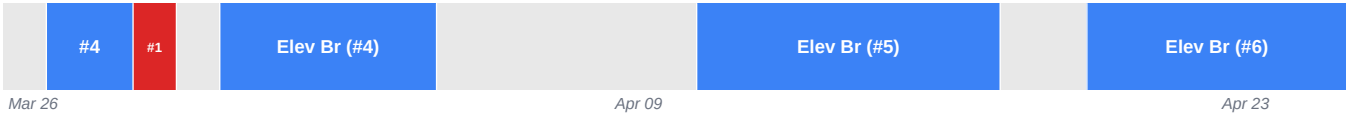
Report Date: April 26, 2024

Coverage: Bed: 552/744h (74.2%)
Chair: 395/744h (53.1%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Patient clinical status over 31 days from Mar 26 to Apr 25

■ HR ■ Breathing



Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
29	35h	Low Heart Rate	<1	100%

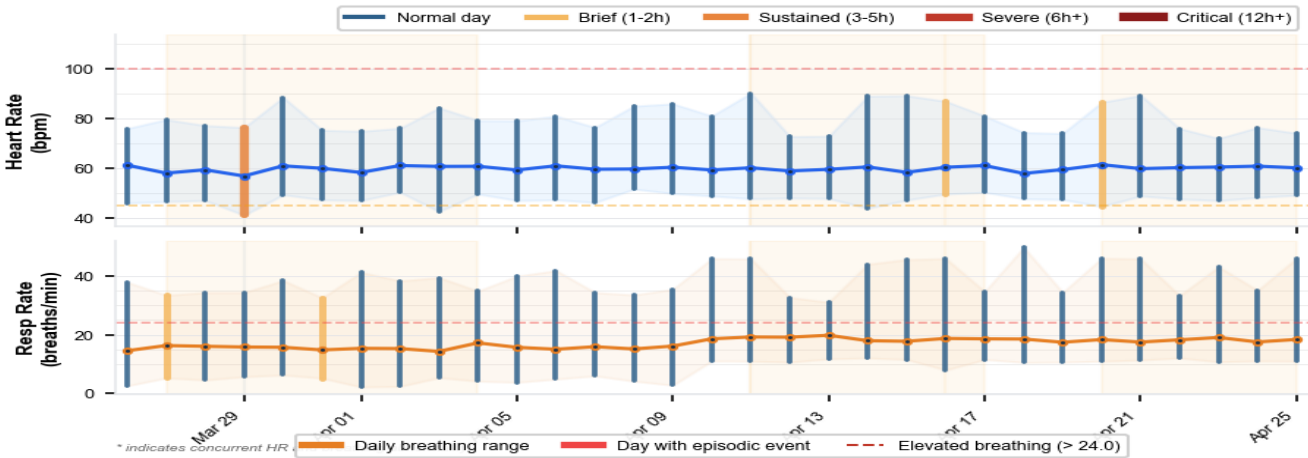
Major Findings: Stable baseline

#	Episode	Min/Max	Total Hours	Longest Continuous	Episodes/d ay	Average	Date
1	Low Heart Rate	41 bpm	3h	2h	2	57 bpm	Mar 29
2	Very High Breathing	46 brpm	1h	1h	1	19 brpm	Apr 16
3	High Breathing	46 brpm	2h	1h	2	19 brpm	Apr 11
4	Elevated Breathing	40 brpm	11h	2h	<1	16 brpm	Mar 27 to Apr 04
5	Elevated Breathing	44 brpm	11h	2h	1	19 brpm	Apr 11 to Apr 17
6	Elevated Breathing	46 brpm	7h	1h	1	18 brpm	Apr 20 to Apr 25

RR values outside physiologic range (6–60 breaths/min).

Clinical Pattern Observations:

- 1. Breathing variability: RR spread 10 brpm (11 to 22).
- 2. Clustered pattern: Episodic events on 5 of 31 days (16%).
- 3. Daytime pattern: Episodes concentrated during waking hours.



Red bars indicate days with detected episodic events.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Clinical Alerting Thresholds

- Very Low HR < 40 bpm
Very High HR > 110 bpm
- Low HR < 45 bpm
Elevated Breathing > 24 brpm
- Elevated HR > 95 bpm
High Breathing > 30 brpm
- High HR > 100 bpm
Very High Breathing > 40 brpm

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.